CAP: advanced salivary gland cancer

National Cancer Control Programme (NCCP) 00615 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests and electrocardiogram (ECG). Heart-pumping assessment with an echo or multigated acquisition (heart pumping) scan (MUGA) scan if over 65 or clinically indicated; doxorubicin requires left ventricular ejection fraction (heart pumping measurement) (LVEF) above 50%. Hearing assessment and measured kidney clearance when indicated.

STEP 2

Taking or receiving treatment

Doxorubicin is given slowly into a vein over about 15 minutes, cisplatin over 2 hours, then cyclophosphamide over 30 minutes. Cisplatin requires prescribed intravenous fluids before and after treatment, so the visit takes longer than the drug infusion times. Tell the nurse immediately about burning, pain, swelling or leakage around the line.

STEP 3

Tests and reviews

Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².

STEP 4

Specific practical instructions

Follow the unit's cisplatin hydration plan and individual fluid target. The source advises 2–3 litres on Day 1 to reduce bladder irritation from cyclophosphamide; the team must adapt this if heart or kidney problems restrict fluids. Report blood in urine or painful urination promptly.

STEP 5

Leaving with a clear plan

Confirm the next appointment, dose, support medicines and verified daytime and out-of-hours phone numbers.

Sources: NCCP 00615 v2: official regimen. Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending.